CHIMALAYA NEPAL

IMPROVING MATERNAL AND CHILD HEALTH

2027 Volunteering Program at Chimalaya Nepal

Information for prospective volunteers

Chimalaya Charity

Chimalaya Charity is a Danish–Nepalese NGO, founded in 2010 by psychotherapist Pia Torp with the purpose of empowering mothers and giving newborns a better start in life by fighting dangerous malnutrition. It has worked in Nepal since 2011, and became a registered NGO in Nepal in 2016.

2010	25,000	~30	~80
founded in Denmark	people in the catchment area	newborns enrolled each month	home visits each month

In 2013, Pia Torp and Chimalaya Charity began working with the doctor and researcher Ram Krishna Chandyo, PhD, and his wife, the paediatrician and researcher Manjeswori Ulak. Both were educated in the West and now work in their homeland to improve the health of the local population. Together, with the help of the local community, they established the mothers' group clinic in Bode, Thimi, just outside Kathmandu.

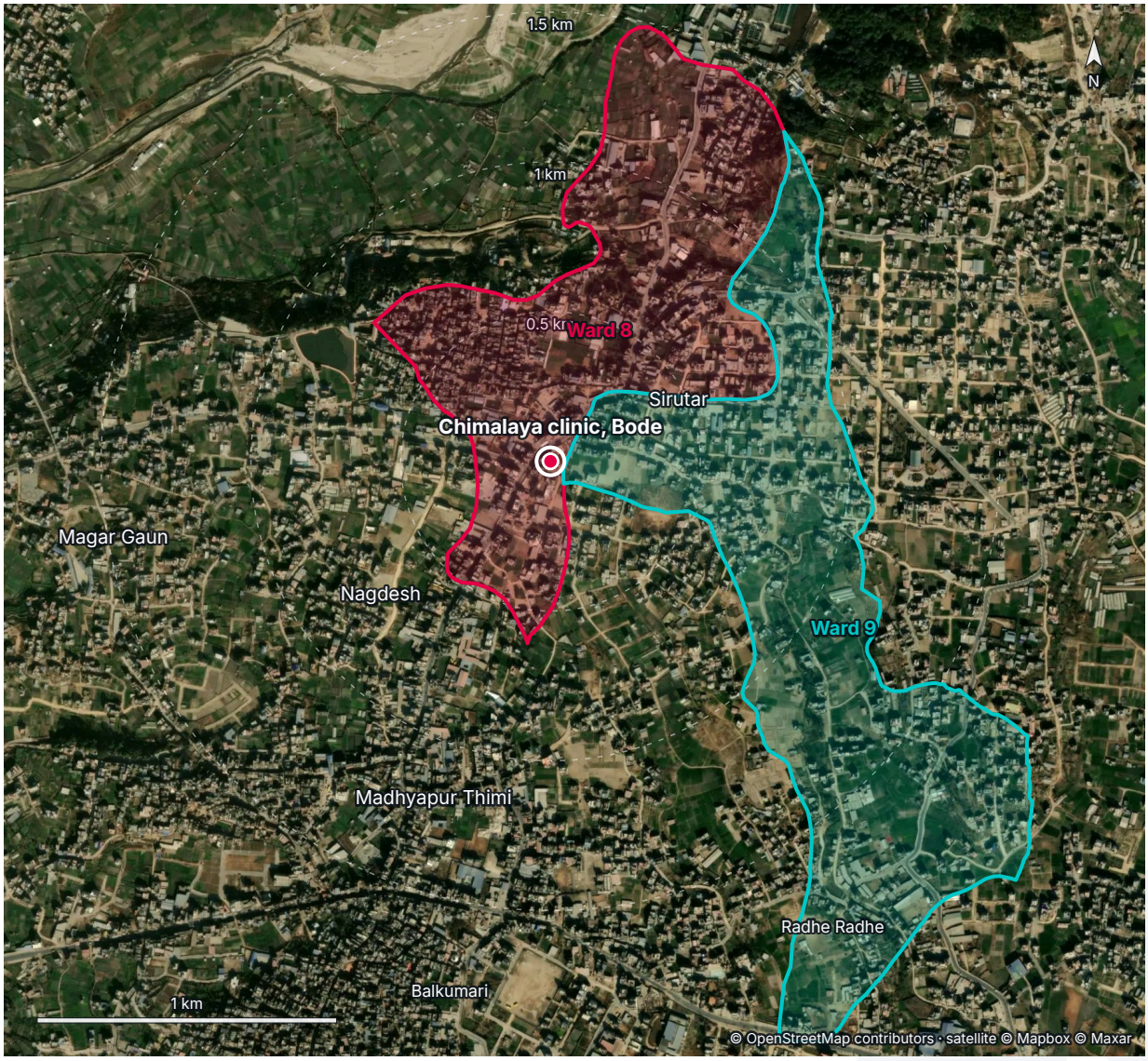
The clinic is the heart of our work. It covers a catchment area of about 25,000 people across wards 8 and 9 of Madhyapur Thimi — an area of small towns and many carpet and brick factories, where women do hard physical labour. For many locals the clinic and its staff are a safe and familiar setting, which makes it easier to reach the most vulnerable families.

Home visiting is one of the clinic's primary activities. Every newborn in the local area is offered a first visit, two further visits during the first month, and a final visit at around six months. Mothers are then invited to join mother-groups until the child is about two years old.

Our mission and reach

With the vision *Improving Maternal and Child Health*, Chimalaya Charity works to combat malnutrition and to promote health and development in children, through the empowerment and education of mothers and their families. We want to secure maternal health before and after birth, the survival of newborns, and the reversal of the negative growth curve in children under five.

Our Nepalese colleagues collaborate with local health authorities and conduct outreach camps several times a month in impoverished mountain areas outside Kathmandu. We also visit brick and carpet factories, where women work with their infants under unhygienic conditions and very poor living standards.



The clinic at Bode, and wards 8 and 9 of Madhyapur Thimi, where Chimalaya works. Dashed rings mark distance from the clinic. Map data © OpenStreetMap contributors, satellite imagery © Mapbox © Maxar.

Our approach combines prevention and health promotion, with a strong focus on:

- **Empowerment** of mothers and families
- **Professional knowledge** and capacity building

- **A sustainable future** for the local community

Our work is always based on local needs. The idea of mothers' groups comes from the Nordic countries, for instance, but at our clinic the model is adapted to Nepalese conditions. The clinic's leading doctors are associated with the Center for International Health in Bergen, which gives the clinic access to current research on malnutrition, and the programme includes ongoing capacity building and training of local health workers.

Clinical activities and programmes

Home visits

The clinic performs approximately 80 home visits per month, following each family through the child's first six months.

Visit	When	What happens
1st	Shortly after birth	General newborn examination (head-to-toe), screening for danger signs, and breastfeeding guidance. **The family is given a Bag4Life bag**
2nd	2 weeks	Checking the well-being of both mother and baby; reinforcing breastfeeding techniques
3rd	2 months	Ensuring exclusive breastfeeding and monitoring motor, linguistic, and emotional development
4th	6 months	Final growth evaluation, guidance on starting solid foods, and an invitation to join the clinic's mother-groups

Bag4Life

Bag4Life is not only a bag. It is the whole package a family receives when a baby is born in the clinic's area: a changing bag, handed over at the first home visit, the visits that follow, and an invitation to join the mother-groups afterwards.

The bag itself holds a towel, a changing mat, reusable nappies, a digital thermometer, cotton wool, yak milk lavender soap and baby massage oil — the last two handmade in Nepal and carrying the clinic's own label. The visits bring a trained health professional into the home to check on, support and advise mother and child — the role a *sundhedsplejerske*, a health visitor, plays in Denmark, carried over to a setting where almost no family would otherwise see one.



A Bag4Life bag, handed over at the first home visit.

Every first visit means handing over a bag, and as a volunteer that will usually be you doing it. You arrive with the bag, go through what is inside with the mother, and explain the visits that follow. It is the opening of almost every relationship the clinic has with a family, and often the reason you are welcomed into the house at all.



A towel, a changing mat, reusable nappies, baby massage oil, yak milk lavender soap, cotton wool and a digital thermometer.

Programmes alongside the home visits

Home visiting is the core of the clinic's work, but not the whole of it. On top of the visits, the clinic runs a set of standing programmes, and volunteers take part in all of them:

Mother-groups

Twice a week · mothers with children aged 6–24 months

- Health and hygiene workshops
- Nutrition courses, taught as cooking demonstrations
- Growth monitoring against the WHO growth standards
- Yoga classes

A safe social network for knowledge sharing and peer support, carrying families on from the final home visit until the child is about two.

Pregnancy groups

Group sessions · 10–12 expectant mothers at a time

- Pregnancy care and what to expect
- Breastfeeding awareness before the birth
- Delivery care and birth preparedness
- Each participant receives a pregnancy care bag

Reaching mothers before the birth means the first home visit is not the first conversation.

School programme

Health education in local schools

- Height and weight measurements
- Dental hygiene
- General health education

Factory visits

Brick and carpet factories around the clinic

- Hygiene measures where families live on site
- Health checks for newborns and young children
- Contact with mothers who cannot easily reach the clinic

Among the highest-risk environments in the catchment area.

Training and capacity building

Strengthening the people who stay on after volunteers leave

- Home visit nurse training
- Female Community Health Volunteer (FCHV) training
- Breastfeeding counselling and parenting education
- Ongoing training of local health workers
- School teachers, to help them spot vulnerable children

Volunteers often contribute to these sessions as well as to visits.

Your role as a volunteer

We welcome volunteers who are either students or qualified professionals within the health-care field, such as:

- Nurses and health visitors
- Midwives
- Physiotherapists
- Nutritionists

Other relevant professional backgrounds are also considered.

> *As a volunteer, you will get straight to the heart of Nepalese homes — and meet some of the warmest, most welcoming people, who despite widespread poverty and limited resources carry themselves with immense dignity, spirituality, and hospitality.*

What you will be doing

You will use your professional skills to promote health and prevent illness among infants and mothers. Most of that happens in people's homes, not in a consulting room.

- **Home visits**, alongside the clinic's nurses — newborn examinations, screening for danger signs, breastfeeding support, growth monitoring, and handing over the Bag4Life bag at every first visit
- **Mother-groups and pregnancy groups**, helping run workshops, nutrition sessions and growth monitoring
- **School and factory outreach**, with measurements, hygiene and health education
- **Training sessions** for local health workers, where your own background is useful

What is expected

- A background, or current study, in a relevant health field. The work is clinical, with real families and real newborns.
- Working **alongside** the Nepalese staff rather than apart from them. They lead the visits, know the families, speak the language, and decide what happens; you contribute to their work.
- Readiness for low-resource settings. You will be in cramped homes and on factory sites, without the equipment you are used to.
- Respect for the families who let you in. Most live with very little and receive you as a guest; the relationship is the reason the clinic reaches them at all.





A mother and her baby with a Bag4Life bag, and a bag being handed over at a visit.

Practical information

Fees

Stay	Fee	Approximately	Included
1 week	25,000 NPR	1,100 DKK / 150 EUR	Placement at the clinic, supervision by clinic staff, and lunch, tea and coffee on working days
1 month	90,000 NPR	4,000 DKK / 535 EUR	As above

> **To be confirmed**
> Left blank deliberately rather than guessed at: the cost of accommodation, what the fee excludes (flights, visa, insurance, local transport), how and when payment is made, whether a deposit is required, and any minimum stay.

Accommodation

The clinic can arrange accommodation, either bed and breakfast or a homestay with clinic staff, which lets you experience the local culture firsthand.



A twin room, bathroom and kitchen, the terrace, and the house itself in Bode.

Contact

To apply, or to ask anything about a placement, write to the clinic:

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